

This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction

Japanese Encephalitis Vaccine (Ixiaro)

Standalone PGD, separated from the combined travel document

This PGD was previously part of a single combined document covering Japanese encephalitis, rabies and meningococcal ACWY vaccination. That was raised by Pharmacy Plus Health on 21 August 2026: a pharmacy adopting one of the three was signing for all three, including vaccines it may not stock and in which its staff may not be trained. The three have been separated so that each can be adopted, trained for and signed off on its own merits.

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPC for the products supplied, as well as BNF advice and any applicable national guidance • All users must be competent in intramuscular vaccination technique and in the recognition and management of anaphylaxis • All users must be competent in travel risk assessment, including itinerary, duration, season and activity • All users must be able to apply the Green Book decision framework for who should and should not be offered this vaccine • All users must previously have used PGD's to supply or administer medication • Must work in compliance with the SOPs of their own employer • All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal • All users must be up to date with MHRA and safety alerts with regards to medicinal products • All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005

Clinical condition or situation to which this PGD applies

PGD Indication	Active immunisation against Japanese encephalitis in individuals at risk through travel or occupation. Japanese encephalitis is a mosquito-borne flavivirus infection of Asia and the western Pacific. Most infections are asymptomatic, but encephalitis carries a case fatality of around 30% and leaves lasting neurological damage in a substantial proportion of survivors. There is no specific treatment, which is why prevention matters.
Inclusion criteria	• Aged 2 months or over. Ixiaro is licensed from 2 months of age

	with no upper limit. • Vaccination recommended: residence in an endemic or epidemic area; a stay of one month or longer in an endemic area during the transmission season; frequent travel to endemic areas; or laboratory work with potential exposure. • Vaccination to be considered: a shorter stay with a higher risk itinerary or activity, for example rural travel, visiting rice fields or pig farming areas, or extensive outdoor and evening exposure; or an uncertain itinerary or duration within an endemic area. • Sufficient time before travel to complete the primary course, ideally with at least one week between the second dose and potential exposure. • Valid informed consent obtained, including consent to any off-label use of the rapid schedule where that applies. • No exclusion criteria present.
Exclusion criteria	• Under 2 months of age. • Confirmed anaphylactic or serious systemic reaction to a previous dose of Ixiaro, or to any component including the residues protamine sulphate, formaldehyde, bovine serum albumin, host cell DNA and protein, and sodium metabisulphite. • Hypersensitivity reaction following the first dose. Do not give the second dose; refer. • Acute severe febrile illness. Postpone until recovered. • Pregnancy, unless the risk of Japanese encephalitis is high and cannot be avoided. Refer for individual assessment rather than vaccinating under this PGD. • A short stay of less than one month confined to urban areas with a low risk itinerary, where vaccination is not recommended. Explain the reasoning and give bite avoidance advice rather than vaccinating.
Cautions	• Breastfeeding. Limited data. Avoid as a precaution unless the risk of exposure is significant, and record the risk assessment. • Immunosuppression. An adequate immune response may not be achieved. Counsel accordingly and consider referral for serology where the risk is high. • Bleeding disorders, thrombocytopenia or anticoagulation. Give by deep subcutaneous injection rather than intramuscularly. • Age over 65. Seroconversion is lower, around 65% compared with over 96% in adults under 50, and titres are lower. Counsel that protection may be less reliable and consider the booster at 12 months. • A history of allergy or urticaria is not in itself a reason to withhold Ixiaro. The Green Book records no known additional risk of hypersensitivity in people with pre-existing allergies. • Vaccination does not remove the need for mosquito bite avoidance, which must be counselled in every case. The mosquito that transmits Japanese encephalitis bites mainly between dusk and dawn.
Anaphylaxis and observation	• Adrenaline 1:1000 injection must be immediately available whenever a vaccine is administered under this PGD, together with a written anaphylaxis protocol consistent with current Resuscitation Council UK guidance. • All staff administering vaccines must be trained in the recognition and immediate management of anaphylaxis and must hold current basic life support training.

	• The patient should remain on the premises for observation after vaccination in line with the pharmacy SOP. • Procedures must be in place to prevent injury from a vasovagal faint, which is common around vaccination, particularly in adolescents.
Actions if patient is excluded or declines treatment	• Discuss the reason for exclusion with the patient and ensure they understand it. • Advise on alternative options and how these can be accessed, for example the GP, a travel clinic, or a specialist service. • Document the reason for exclusion, the advice given and the decision reached. • Inform or refer to the GP as appropriate, and where the exclusion is time critical make the urgency of that referral explicit to the patient.

Details of the medicine

Name, form and strength of medicine	Ixiaro (Valneva), suspension for injection in a pre-filled syringe. Inactivated, adjuvanted, Vero cell derived, strain SA14-14-2. This is the only Japanese encephalitis vaccine licensed in the UK. Record the batch number and expiry for every dose.
Legal category	POM
Storage	Store at 2°C to 8°C in the original packaging, protected from light. Do not freeze, and discard any vaccine that has been frozen. Shake before use to obtain a homogeneous white suspension; a fine white deposit with a clear supernatant on standing is normal.
Route/method of administration	• Intramuscular injection. Deltoid in older children and adults, anterolateral thigh in infants. • In bleeding disorders, thrombocytopenia or anticoagulation, give by deep subcutaneous injection instead. • May be given at the same visit as other travel vaccines at a separate site, preferably a different limb, or at least 2.5 cm apart in the same limb. Co-administration with inactivated hepatitis A vaccine and with rabies vaccine has been studied without interference.
Dose and frequency of administration	• Aged 3 years and over: 0.5 mL per dose. • Aged 2 months to under 3 years: 0.25 mL per dose. Follow the SPC instructions for discarding the excess volume using the marking on the syringe, and use a new sterile needle for the injection. • Conventional primary course, all licensed ages: two doses, day 0 and day 28. • Rapid primary course: two doses, day 0 and day 7. This is licensed only for adults aged 18 to 64. Its use in children and in adults aged 65 and over is off-label; the Green Book permits it where there is genuinely insufficient time before travel, and off-label use requires explicit documented consent. • Complete the primary course at least one week before potential exposure. • If the course is interrupted, resume it rather than restarting. A second dose given up to 11 months after the first still produces a high rate of seroconversion. • First booster: 12 to 24 months after the primary course and before re-exposure. Those at continuous risk, such as long-term residents and laboratory staff, should have it at 12 months.

	Second booster: adults aged 18 to 64 at continued risk, 10 years after the first booster. Long-term data are lacking in children and in the elderly, in whom immunity may wane sooner.
Quantity to be administered and/or supplied	One dose per patient per attendance, in accordance with the schedule above.
Adverse effects	Very common: headache, myalgia, injection site pain or tenderness, and fatigue. Common: nausea, injection site redness, induration, swelling and itching, influenza-like illness and fever. Uncommon: lymphadenopathy, migraine, dizziness, vertigo, gastrointestinal upset, rash, pruritus, arthralgia, chills, malaise and raised hepatic enzymes. Rare: thrombocytopenia, paraesthesia, neuritis, syncope, palpitations, tachycardia, dyspnoea, urticaria and peripheral oedema. In children, fever, diarrhoea, influenza-like illness and irritability are the most frequent. Consult the current SPC for the full list.
Yellow card reporting	Report suspected adverse reactions via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: - that valid informed consent was given - name of individual, address, date of birth and GP with whom the individual is registered - name of healthcare practitioner - name and brand of vaccine, batch number and expiry date - date of administration, dose, route and anatomical site used - where a course is involved, the dose number and the date the next dose is due - advice given, including advice given if excluded or if the patient declines - details of any adverse reaction and the action taken - that the vaccine was administered via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. The individual's GP should be informed.

Patient information

- Give the patient the manufacturer's patient information leaflet.
- Explain the schedule clearly, including the date the second dose is due and that the course must be completed at least one week before exposure.
- Counsel on mosquito bite avoidance as the primary protection: repellent containing DEET, covering up at dusk and dawn, and treated bed nets. Vaccination supplements this and does not replace it.
- Explain that no vaccine is completely protective and that any febrile illness with headache, confusion or neurological symptoms during or after travel needs urgent medical assessment, mentioning the travel history.
- Counsel on the common local and systemic reactions.
- Where the rapid schedule has been used off-label, record that this was explained and consented to.

References

- UKHSA. Immunisation against infectious disease, the Green Book, chapter 20, Japanese encephalitis. Revised 6 February 2024.
- Summary of Product Characteristics for Ixiaro, current version, electronic Medicines Compendium.
- NaTHNaC TravelHealthPro. Country information pages and Japanese encephalitis disease page.
- Resuscitation Council UK. Emergency treatment of anaphylaxis, current guidance.

Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates	

Name of healthcare professional	Job title	Registration number	Signature

Valid from date	Expiry date

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or vaccine.

To be completed by the adopting pharmacy: the first block is signed by that pharmacy's superintendent or clinical lead, and the second by its professional group signatory. Get Real Health signs the authorship block below, not these.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Name of professional group signatory	Job title & name of organisation	Signature	Date

Signed on behalf of Get Real Health

Name	Qualification	Signature	Date
Nitin Shori	Medical Doctor GMC: 6047293		21/08/2026
Chris Pilkington	Pharmacist GPhC: 2046322		21/08/2026

Change history

Version number	Change details	Date
001	New standalone PGD. Separated from the combined Japanese encephalitis, rabies and meningococcal ACWY document	21st August 2026

	following the query from Pharmacy Plus Health on 21 August 2026. Content rebuilt from Green Book chapter 20 and the current Ixiaro SPC. Notable additions the combined document did not carry: the paediatric half dose of 0.25 mL below 3 years, the restriction of the day 0 and day 7 rapid schedule to adults aged 18 to 64 with off-label use elsewhere, the deep subcutaneous route in bleeding disorders, and the reduced seroconversion above 65.	